



PT Name : MR. MITHUN	Age : 25 Y Gender : MALE	SAMPLE COLLECTED AT : RAVI DIAGNOSTIC & SCAN CENTRE
Ref By Dr : SELF	Registered on : 21-05-2026 10:55 AM	
Booking Id : 942314 HPLTS266 - 2	Received on : 21-05-2026 11:41 AM	
Barcode Id : HED198642	Reported on : 21-05-2026 01:31 PM	SAMPLE : EDTA
INV : GLYCATED HAEMOGLOBIN (HBA1C)		

Glycated Haemoglobin (HbA1c)

TEST DESCRIPTION	RESULT	UNITS	BIO. REF. RANGES
HBA1C Method: Fully Automated H.P.L.C.	5.1	%	0 - 6.5 Non diabetic adult >=18 years <5.7 At risk (Prediabetes) 5.7-6.4 Diagnosing Diabetes >= 6.5
Estimated Average Glucose Method: CALCULATED	99.67		

Reference Range: As per ADA Guidelines

Below 5.7% : Normal
5.7% - 6.4% : Prediabetic
>=6.5% : Diabetic

Guidance For Known Diabetics

Below 6.5% : Good Control
6.5% - 7% : Fair Control
7.0% - 8% : Unsatisfactory Control
>8% : Poor Control

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

HbA1c is what's known as glycated haemoglobin. This is something that's made when the glucose (sugar) in your body sticks to your red blood cells. Your body can't use the sugar properly, so more of it sticks to your blood cells and builds up in your blood. Red blood cells are active for around 2-3 months, which is why the reading is taken quarterly.

A high HbA1c means you have too much sugar in your blood. This means you're more likely to develop serious problems with your eyes and feet. Knowing your HbA1c level will help you reduce your risk of devastating complications. This means getting your HbA1c checked regularly. It's a vital check, and you're entitled to get this test at least once a year. But if your HbA1c is high or needs more attention, it'll be done every three to six months.

~~End of report~~

D. Venkateshwarlu
Manager Technical

Dr. Vinay Kumar
MD PATHOLOGIST



PT Name : MR. MITHUN
Ref By Dr : SELF
Booking Id : 942314 HPLTS266 - 2
Barcode Id : HED198641
INV : KIDNEY PROFILE

Age : 25 Y | Gender : MALE
Registered on : 21-05-2026 10:55 AM
Received on : 21-05-2026 11:42 AM
Reported on : 21-05-2026 04:01 PM

SAMPLE COLLECTED AT :
RAVI DIAGNOSTIC & SCAN CENTRE
SAMPLE : SERUM

Kidney Profile

TEST DESCRIPTION	RESULT	UNITS	BIO. REF. RANGES
Urea Method: UREASE-GLDH	37.77	mg/dl	18 - 55
CREATININE Method: Enzymatic	1.10	mg/dl	0.6 - 1.3
BUN/Creatinine ratio Method: CALCULATED	16.05		8.5 - 23.5
BUN (Blood Urea Nitrogen) Method: CALCULATED	17.65		7 - 25
Uric Acid Method: URICASE	7.2	mg/dl	3.5 - 7.2
Calcium Method: ARSENAZO	8.98	mg/dl	8.6 - 10.2
SODIUM Method: ISE-Electrode Direct	140.28	mEq/L	136 - 145
POTASSIUM Method: ISE-Electrode Direct	4.68	mEq/L	3.6 - 5.1
CHLORIDE Method: ISE-Electrode Direct	100.45	mEq/L	98 - 107

INTERPRETATION:

Kidney function tests are urine or blood tests that evaluate how well your kidneys are working. Most of these tests measure glomerular filtration rate (GFR). GFR assesses how efficiently your kidneys clear waste from your system. They help your body filter waste materials and expel them as urine. Your kidneys are also vital for producing:

- Hormones that maintain blood pressure.
- Red blood cells, which carry oxygen throughout your body.
- Vitamin D, which maintains bone and muscle health.

~~End of report~~

D. Venkateshwarlu
Manager Technical

Dr. Vinay Kumar
MD PATHOLOGIST



PT Name	: MR. MITHUN	Age	: 25 Y Gender : MALE	SAMPLE COLLECTED AT :	
Ref By Dr	: SELF	Registered on	: 21-05-2026 10:55 AM	RAVI DIAGNOSTIC & SCAN CENTRE	
Booking Id	: 942314 HPLTS266 - 2	Received on	: 21-05-2026 11:42 AM		
Barcode Id	: HED198641	Reported on	: 21-05-2026 04:51 PM	SAMPLE : SERUM	
INV	: THYROID STIMULATING HORMONE (TSH)				

Thyroid Stimulating Hormone (TSH)

TEST DESCRIPTION	RESULT	UNITS	BIO. REF. RANGES
Thyroid Stimulating Hormone (TSH) Method: Electrochemiluminescence Immunoassay 'ECLIA'	1.66	µIU/mL	1day - 30days :0.52-16.0 µIU/mL 1month - 5yrs :0.46-8.10 µIU/mL 6yrs - 18yrs :0.36-5.80 µIU/mL 18yrs - 55yrs :0.35-5.50 µIU/mL 56yrs - 101yrs :0.50-8.90 µIU/mL

Pregnancy Ranges :

Ist Tri : 0.1-2.5

IIInd Tri: 0.2 - 3.0

IIIrd Tri : 0.3 - 3.1

Interpretation:

In primary hypothyroidism, thyrotropin (TSH, formerly thyroid-stimulating hormone) levels will be elevated. In primary hyperthyroidism, TSH levels will be low.

The ability to quantitate circulating levels of TSH is important in evaluating thyroid function. It is especially useful in the differential diagnosis of primary (thyroid) from secondary (pituitary) and tertiary (hypothalamus) hypothyroidism. In primary hypothyroidism.

TSH levels are significantly elevated, while in secondary and tertiary hypothyroidism, TSH levels are low or normal.

Elevated or low TSH in the context of normal free thyroxine is often referred to as subclinical hypo- or hyperthyroidism, respectively.

Thyrotropin-releasing hormone (TRH) stimulation differentiates all types of hypothyroidism by observing the change in patient TSH levels in response to TRH.

For diagnostic purposes, the results should always be assessed in conjunction with the patient's medical history, clinical examination, and other findings

~~End of report~~

D. Venkateshwarlu
Manager Technical

Dr. Vinay Kumar
MD PATHOLOGIST